SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

TPF induction: docetaxel, cisplatin and fluorouracil

Head and neck cancer: induction chemotherapy · NCCP 00315 · version 7a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This chemotherapy is given before definitive treatment to shrink or control the head and neck cancer. The team explains how induction fits your overall treatment aim and why this particular schedule is appropriate.

Schedule
Docetaxel 75 mg/m² and cisplatin 75 mg/m² are given on Day 1. Fluorouracil 1,000 mg/m² per day runs continuously over 96 hours on Days 1–4; the source also allows a separate 22-hour infusion on each of those four days. Induction repeats every 21 days for 3 planned cycles unless cancer progression or unacceptable side effects require a change.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Docetaxel
TAXANE CHEMOTHERAPY

- ☐ Low blood counts, infection and sepsis
- ☐ Hair loss, mouth soreness and fatigue
- ☐ Numbness / tingling; sometimes lasting
- ☐ Fluid retention and nail changes
- ☐ Allergic infusion reaction

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Nausea and vomiting
- ☐ Kidney injury and mineral changes
- ☐ Hearing loss or tinnitus, which can persist
- ☐ Nerve damage, which can persist
- ☐ Infusion reaction and effects on fertility

Fluorouracil
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Mouth and throat soreness
- ☐ Diarrhoea and dehydration
- ☐ Painful palms or soles
- ☐ Heart problems including chest pain
- ☐ Rare severe neurological or early excessive toxicity

Discuss the individual aim, expected benefit, alternatives and uncertainty, including the role of induction before definitive treatment. Explain serious infection and sepsis, low blood counts and bleeding, nausea and dehydration, mouth and bowel toxicity, hair and nail changes, infusion allergy, fluid retention, lasting kidney/hearing/nerve damage, fluorouracil heart or early severe toxicity despite DPD testing, fertility and pregnancy, line/pump risks, steroid and growth-factor plans, and treatment burden. Later surgery, radiation or chemoradiation needs its own consent discussion.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00315 v7a.

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